Breathe Kritisk: Ambiguous control to materialise entangled agency between human, body and technology

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Abstract

This paper presents *Breathe Kritisk,* an inflatable installation that explores the negotiation for control among clinical practitioners, clinical apparatus, and patients in medical settings. The installation aims to materialize the patient's experience of uncanniness and entanglement in agency at the encounter with biomedical technologies. It seeks to provoke visitor reflection on their personal experience and discussion among critical medical humanities and human-computer interaction (HCI) communities.

This article presents my process of creating a functional prototype that manifests a tangible experience of entangled control with an artificial self. The installation results from my practice-led research, which applies contesting control — a technique of estrangement in interaction design. The piece focuses on creating ambiguous control by switching between giving users control and giving it away to machines and internal organs that users do not have complete control over. Through interaction tests, I observed that this installation aesthetically constructed a poetic interaction, where users could not tell apart when they were in control of the inflatables and when they were not. Within this liminal territory, ambiguous control produces a never-ending grappling for control. Lastly, I reflected on ambiguous control as a framework for reflection and materialization entangled agency for practitioners working closely with tangible, participatory experiences.

Keywords: Interactive installation, Poetic interaction, Contesting control, Ambiguous control

Introduction

Breathe Kritisk is part of a practice-led PhD research project, inspired by critiques of how patients have lost and continue to lose control of their bodies to the medical gaze. The work reflects on the ambiguity of patients' agency and control within the clinical hierarchy that is constantly negotiated among clinical practitioners, clinical apparatus, and patients in medical settings. This installation focuses on the patient's experience of encountering artificial self-representation through biomedical devices and two experiential dimensions: uncanniness and entangled agency.

My practice during the PhD engaged with *contesting control* as a framework, which serves as a lens for philosophical engagement and also a foundation/departure point for materialising the installation experience. In this presentation, I described elements and rationales for the development of a prototype's physical construction and programming, as well as participant experience and meaning-making based on the framework.

The digital clinical gaze

There is a hierarchy: in which medical professionals, medical spaces, medical practices and medical devices have authority over patients. Patients surrender the agency of their bodies in clinical circumstances (Boyer, 2019; Sontag, 2002). Patient bodies become a site of biological, psychological or societal illness. The bodies are the host of the "wrongness" that needs fixing (Foucault, 2002). Patient bodies are mechanical objects to be studied by the

clinical gaze, like an engine where technicians investigate technical malfunctions (Tybjerg et al., 2016). There is a conflict between the ideology of patients' autonomy and patients' consent to clinical practices.

Patients' autonomy

Despite endeavours to encourage an ideology of patients' autonomy, their agency remains ambiguous. A contemporary medical practice includes *informed consent,* when clinical professionals explain medical rationales and procedures so that patients can accept or reject clinical evaluation and/or treatment. The medical consent aims to empower patients to make decisions (Paterick et al., 2008), and attempts to formalise and legalise patients' agency (Maclean, 2009). Yet, a systematic review illustrated that, although patients comprehend the informed consent process, they still have limited understanding of their disease, potential complications, and clinical risks (Falagas et al., 2009). The consent is critiqued to be merely a relational concept of autonomy, and medical knowledge remains a gap for patients to overcome to achieve active engagement in medical consent (Stoljar, 2011).

Another endeavour is for patients to position themselves as active patients, educating themselves and establishing interprofessional collaboration (Towle et al., 2010). Multiple trials were conducted for patients with AIDs and cancer (Barbot, 2006; Bishop & Yardley, 2004). Nevertheless, the orthodox within healthcare persists. The established practitioners' language and communication practice make it difficult for patients to critically question the expert recommendations and the proposed consents. When cancer patients attempt to discursively re-position themselves as active agents, they are faced with an ideological dilemma to subvert the hierarchy. As they had to claim expertise to justify their reasoning with the clinical practitioners, it resulted in emotional dissonance (Bishop & Yardley, 2004). Thus, it remains a paradox which constantly demands patients to submit, surrender and obey, taking the bodies with illness as hostage – an invincible negotiation between surrender or suffer (Boyer, 2019, p. 85).

Uncanniness

Sigmund Freud articulated uncanniness as the feeling of being 'exposed to a personal, intimate, private side of selves' (Freud, 1919). He pointed to a German word, "unheimlich" and its opposite term, "heimlich" for clarification. Heimlich could be translated as homey; belonging to the house; intimate; friendly; familiar, and it could also be defined as concealed; private; secretive; withheld from sight and from others; deceitful. (Notably, heimlich includes the notion of being concealed from oneself.) Resonating on the opposite side, unheimlich is defined as unhomey; unfamiliar; uncomfortable; eerie; weird and it is also unconcealed, unsecret; what is made known; what is supposed to be kept secret but is inadvertently revealed (Gray, 1989). For such semantic psychoanalytic, Freud's thesis is that the eerie, uncanny feeling comes from a self-exposure or self-revelation of what is private and concealed, of what is hidden; hidden not only from others, but also from the self.

I projected this definition onto medical bodily representation – medical technologies, biosensors and algorithms expose patients' bodies in ways which sometimes evoke uncanny feelings. My PhD supervisor, Anders Sundnes Løvlie, reflected on his personal experience, looking at his beating heart on a screen. He didn't feel at home with his body on display, although his body was supposed to be his home. He struggled to connect emotionally and viscerally to what he was told was him. For such phenomena, physiological sensors and their representation meddled with our sense of "what is me?" and "how do I feel me?"

(Schwartzman, 2011). Although medical technology is built to give us access to connect to our inner selves, it alienates us from feeling at home and in control of our bodies.

Entangled Agency

Biodetection with digital representation transforms us into a *datafied cyborg* and a *quantified self* (Lupton, 2012, 2016). A continuous glucose monitor attached to the user's forearm, allows people with diabetes to measure and keep track of their blood glucose level and regulate it with an automatic insulin pump. In one way, this datafication practice flattens out the hierarchy, empowering users without medical knowledge to access their internal body signals, become more aware of the inner state of their bodies, and care for themselves (Homewood et al., 2021).

Yet, this could give a wrong sense of control and agency. Users with diabetes change the batteries of the insulin pump and continuous glucose monitor, so the devices can in return help them to regulate their bodies. Bodies are made reliant on these biosensors for self-regulation and are inseparable from digital materials. For such negotiation, devices turn their users into *disabled cyborg bodies* (Forlano, 2017).

Biosensors dissolve the boundary of the body between subjects and objects (Schwartzman, 2011). The hierarchy between human and medical devices is dynamic, material, relational and being reconfigured at all times (Boyer, 2019; Homewood et al., 2021). For this broken boundary, patients grapple to find the grounds and agency of their gazed bodies. As Han Kang put it in her novel, *Vegetarian* (Kang, 2016);

> *'It's your body, you can treat it however you please. The only area where you're free to do just as you like. And even that doesn't turn out the way you wanted.' – Han Kang (2016)*

Contest control

Research in Human-Computer Interaction (HCI) has explored the design of *uncomfortable interactions*, which intentionally cause a mild degree of suffering to users (Benford et al., 2012). *Discomfort through control* is one of four depicted principles to create uncomfortable experience (Benford et al., 2012). The centre of the discomfort-through-control principle is to reverse the user-friendly fundament of giving users control over the interface. The principle suggests taking away control from direct users and, instead, providing unusual control to other users or machines. This reversal of control can induce discomfort by asking the users to surrender, to lose control, or to battle for control. This discomfort could be the source of reflective experience for participants.

The combination of discomfort through control and the physiological sensors for partially autonomic bodily activities (for instance, breathing, blinking, and heart beating) can lead users to battle/contest to stay in control of both the system and their bodies and thoughts (Benford et al., 2021). When *contesting control*, users travel through various experiential dimensions. Three experiential dimensions were depicted: (1) aware-of-control and not-aware, (2) in-control and not-in-control, and (3) tight-control and loose-control. The state of *ambiguous control* is located among all those dimensions. It is a stage where users travel past, but not necessarily stay in. Furthermore, its definition or description of the stage is not elaborated or clarified. It was vaguely implied to link with unpredictability, unstable, and uncontrollability (Benford et al., 2021).

On a related artistic practice of creating interactive arts, Simon Penny theorises *poetic interaction* as the interactivity with aesthetically behaving systems (Soler-Adillon, 2015 citing Penny, 2011). Aesthetic behaviours are directed toward unpredictability – when interactive systems are neither easy to control nor easy to predict (Soler-Adillon, 2015). "It should not be predictably instrumental, but should generate behaviours which exist in the liminal territory

between perceived predictability and perceived randomness. A zone of surprise, of poetry" (Penny, 2011). It is a dance between expectations and surprise, where the interactive artefact seeks to engage the user in exploring and experimenting with it; a dance of agency (Pickering, 1993). Simon Penny reflected on his 20 years in the new-media-art community. He remarked on the limited research on qualitative discussions of aesthetically rich interaction and the complexities of designing interaction as an artistic practice .

Picking up from the traces of ambiguous control and poetic interactions, this paper investigates in visitors' embodied experience (Dewey, 1958) of the ambiguous control and if the experience could be a discursive resource to instigate discussion and deepen the comprehension of the experiential complexity of patients, subjected to medical practices (Tharp & Tharp, 2019). This paper hopes to contribute to such a discussion through practice-led research methodology.

Method

My practice follows an iterative design convention of building, testing and iterating with participants in a university laboratory. I took notes on my creation process and design decisions as I explored how to create the experience of ambiguous control with physiological sensors. Below, I describe the process of developing a functional prototype (rather than the latter stage of production), because it captures deliberate selections of artefact elements that lead to poetic interaction between users, user bodies and machines.

In order to capture the emotional and bodily experience of the interaction, I conducted phenomenological interviews, a qualitative approach aimed at understanding a user's lived experiences (Bevan, 2014). I chose the qualitative approach to gain in-depth comprehension of each participant's experience in their worlds. I adopted an imaginative variation: giving space for the participants to give free association and description of their experience and asking follow-up questions to add clarification. Then, I asked participants if they have any association with medical or clinical experiences.

I tested and collected impressions from 20 participants in the university laboratory. Their backgrounds include interaction design or HCI (n=10), computer science (n=3), art historian (n=2) and medical humanities (n=5). I recorded and made notes of their responses and photographed their interactions.

My data analysis focused on how participants experience the ambiguous control of the functional prototype and whether the experience could provoke users to critically reflect on the clinical practices. At the end, I reflected on my practice in working with ambiguous control. This includes values on ambiguous control throughout the creation and research process; an experiential core of ambiguous control through participant reports and observation; and its potential and implications to provoke visitor reflection.

Creation process

I sought to produce the uncanniness and entanglement that patients experience at their encounter with clinical self-representation – an experience that ambiguates a sense of self-control and body agency – to challenge the ideology of patients in control of their decisions and bodily agency within the medical practice. Based on the concept of contested control, I aspired to create a physiological-sensor system that partially gives control to the users and, at unpredictable times, reverses that control.

Breathing and physical construction

The first step of the creation process is the selection of involuntary physiologic processes for users to interact with. I considered partially voluntary bodily functions

(heartbeats, breaths, brain signals, and palm sweat) in relation to available and hackable biosensors (heart-rate sensor on a fingertip, EEG and galvanic skin response sensor). Breath is one of the bodily behaviours most extensively explored by HCI design researchers, due to the fact that users can be aware of their breath and control it to some certain extent, as well as its significant impacts on users' mental, physiological and sociological status (Aslan et al., 2016; Fritsch et al., 2025; Karpashevich et al., 2022; Sakurai et al., 2015; Shamekhi & Bickmore, 2018; Ståhl et al., 2016; Tsaknaki, 2021; Wernli & Boyer, 2021; Yu et al., 2015). While profuse works aim to support users to connect to their breaths (i.e., meditating exercises and emotional regulations), I aim to reverse this techno-deterministic goal and to create an uncanny interaction with breaths.

The spatial construction of the installation was based on my initial idea of *externalizing* user lungs (Schnädelbach et al., 2010). This followed the notion of the unheimlich (Freud, 1919) of making one body's into a home, but an un-homey home. A chamber construction was intended as walls of a home: a skin wall of a body, made in a 90-degree-angle, geometric way. The chamber was enclosed from three sides, wishing to give a sense of partial privacy in a medical setting. Within the chamber, two air pillows breathe inside, like mechanic lungs. To intensify the un-homeliness, I aimed at making a twisted version of user's lungs (visual references in Figure 4, sketch in Figure 5)



Figure 4: Clinical rooms, medical apparatus and clinical depictions of diseased lungs

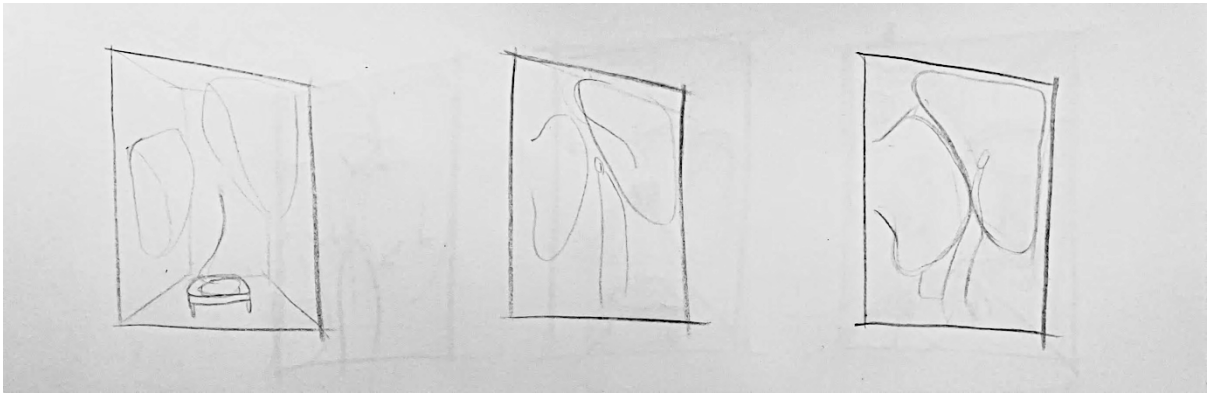


Figure 5: Early sketch of the spatial construction and its interaction

Air pillows were made from parachute fabric, whose mesh captures wind but is not completely airtight. The air leakage by fabric option was selected for safety reasons: to limit the force air pillows put on interactor bodies and to avoid bursts.
Two air systems were tested: fans and an air compressor. Fans intended to give the air volume inside the air pillows and the air compressor intended to act as a disrupting disease, creating air pockets in unexpected areas and moments (Figure 6).
Breath sensors often come in the form of strap sensors (a belt band that goes around the user's chest or stomach – capturing the change of user's body shape for inhales and exhales) or mouthpiece sensors (a sensor to put around the user's mouth or nose to detect air coming in and out). The strap sensor was selected because it would be more convenient to keep hygiene. The strap sensor used a thin film pressure sensor touching the user's stomach – with an assumption that their inhaling body will push into the force sensor, increasing the detected force, and their exhaling body will withdraw from the sensor, decreasing the detected force.
In the early phase of tests, I found that the force sensor is sensitive to small stomach movements, and this sensitivity interrupts the mirroring flow. For example, when users' stomachs make a mini and unconscious move outward at the most extreme exhale, the fans are activated when they should not (Figure 6). I noticed that the sensor worked much smoother with bigger bodies and users skilled at stomach breathing. Despite my several attempts to write buffering codes to make the interaction as smooth and responsive as possible, it still required careful and deep breathing. When tested, participants said that the body contact with the strap intuitively nudged them to pay attention to their breathing, fitting to the intention for users to pay attention to their body and its connection/disconnection with the machine. Thus, I decided to continue with this strap force sensor and embrace the sensor sensitivity as part of the experience.

For the air system, I decided to only use the fan and discard the air compressor. Despite its interesting effect of twitching and changing the shape of air pillows (Figure 6), the valves for controlling the air compressor made abrupt, loud 'pop' sounds distracted participants from focusing on their connection with the machine. Some participants reported the loud pops as strong, dominant and scary. The sounds took away the subtle uncanniness, the unsaid, the quiet creep, the nuanced satire and humour, which I tried to achieve.

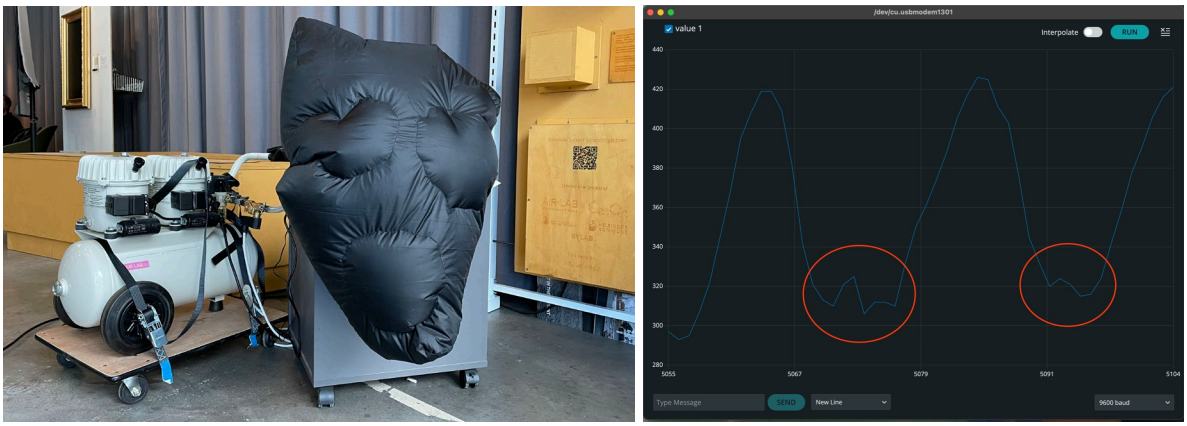


Figure 6: Tested air pillow with fans and an air compressor (the three sub-pockets were pumped by the air compressor, changing the overall shape of the air pillow) and the measured breathing rhythms and unconscious stomach movements (in circles) at the exhales.

Programming the interaction

Visuotactile synchrony, or the projection of one's body to an external representation, can cause people to experience strange visceral "tricks". For instance, the rubber hand

experiment tricks a participant to move away their physical hand when a rubber hand is hammered (Rohde et al., 2011). The mirror therapy helps a patient relieve an imaginary itch on their amputated limb by scratching their other limb in a mirror (Kim & Kim, 2012). These examples work in two stages: first inducing a feeling of body ownership onto a fake/counterfeit body part, then introducing a stimulation on the counterfeit to create an illusion. The experience of this illusion can be highly uncanny. To bring about a similar uncanniness, I programmed two functions: *mirror* and *self-breathe*.

The mirror function controlled the fans following the sensor detection of the user's breaths. If the user's stomach increasingly touched the force sensor (by inhaling in case of stomach breathing), the fans were turned on and the air pillows inflated. If the stomach touch decreased (by exhaling), the fans were turned off and the air pillows deflated.

The self-breathe function controlled the fans based on pre-scripted time, regardless of the input force sensor. For example, the function switched between 3 seconds of turning on the fans and 3 seconds of turning off the fans – the air pillows inflated and deflated as if the machine was breathing by itself. This self-breathe function varied based on a few variables; the time duration of turning the fans on/off, the side of fans that is turned on/off, and the time duration of the self-breathe function.

In order to create entanglement in control, the breathing machine was scripted to alternate seamlessly between mirror and self-breathing functions. In other words, I intentionally made the system waver between responsive actuation (following users' control) and hard-coded actuation (following a program script). The functions switch back and forth without notifying the users in varied periods (for example, 30 seconds of mirror, 20 seconds of self-breathe, then 45 seconds of mirror, ...).

In the middle phrase of creation, I explored several ways how the system reacts to the measured force to match with users' breaths, how to work with the sensor sensitivity, how to code for an ambiguous yet clear interactivity, and how to instruct users practically. From testing with different bodies, I found that the first stage of self-connection with the machine was essential for users to make a distinction between controllable and non-controllable. The responsiveness or the real-time reaction of inflation and deflation based on the user's breaths was a crucial element. If this connection was not made clear to the user, once the self-breathe function is activated the user would assume that the installation malfunctioned and the whole interaction may seem meaningless. For that reason, I iteratively refined the codes and the physical air pillows to inflate and deflate in the right rhythms.

Through this process I have developed a functional prototype of the installation, called *Breathe Kritisk* (Figure 7). The installation switches between following user breaths (to create body ownership/connection) and breathing by itself (possibly to induce the users to follow the machine's breaths).

It should be noted that the transition between the mirror and the self-breathe functions was intentionally made unclear. The system does not give signals to users when it switches between the two functions. An early participant suggested making the visual distinction between the two functions (for example, lights, graphs or numbers), so a user knows right away when they are influenced by the machine. But I decided against the suggestion and remained the ambiguity to give space for visitors to play, search into, and make sense of the ambiguous control.

Final prototype

Breathe Kritisk stands as a two-meter-tall chamber with two air pillows inside. It has an engineer, utilitarian aesthetic. A visitor is invited to put on a breath sensor that straps around one's stomach. The visitor can inflate and deflate the air pillows through the breathing sensor.

The machine sometimes complies with visitors' breaths and other times has its mechanic breaths.

As I welcomed visitors, I gave the following introduction: 'This is Breathe Kritisk. It is a piece about the patients' control in a medical setting. It aims to ask if you control the machine, or if the machine controls you.' Then, I put on the breathing strap under any thick clothing (for example, a sweater), and I informed the visitors about their interactions: 'The sensor is quite sensitive, so you have to breathe in deeply and talking could interrupt its measurement.' In the first 30 seconds, the air pillows would inflate and deflate in response to forces between the user's stomach and the sensor [mirror function]. They may inflate and deflate consistently or inconsistently (with split-second pauses once in a while, due to the sensor sensitivity). Then, the next 15 seconds, the air pillows would be fully and consistently inflated and fully deflated, regardless of the user's breathing [self-breathe function]. Then, the system switches back and forth between the two functions.

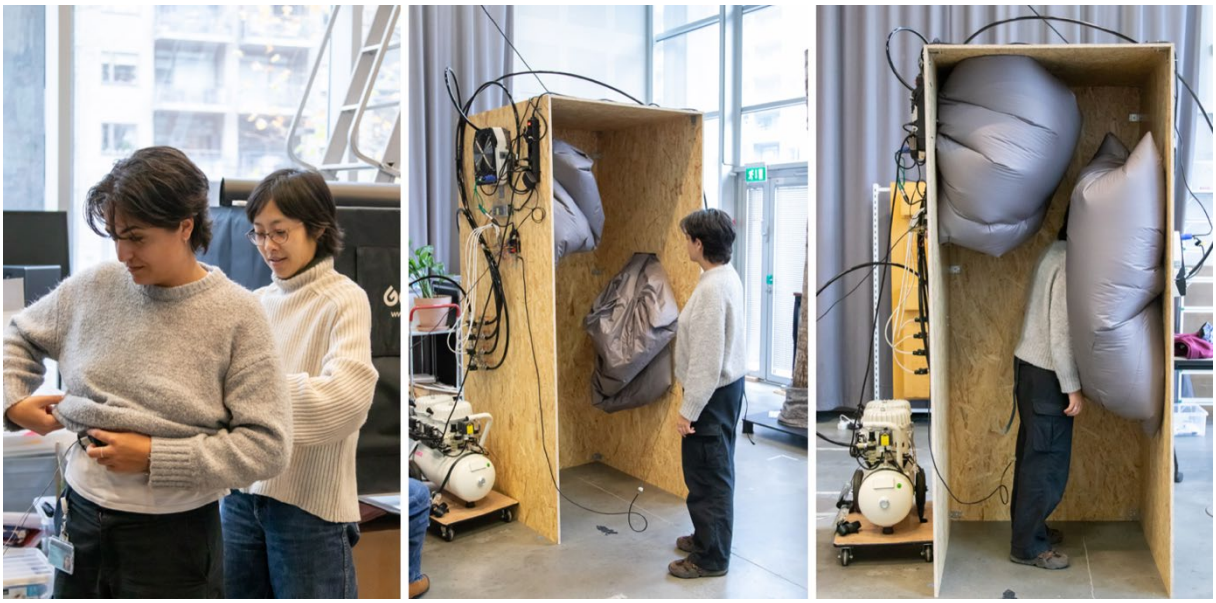


Figure 7: Visitor experience with the functional prototype (putting on the breathing strap and exploring their way of interaction)

Experience of the prototype installation

Ambiguous control

Participant's journey often started from the participant's focus on "How do I and my body control the machine?" or "How do the machine and the sensor work?" Many challenged the system by manipulating their breaths, for example, holding their breaths to see if the machine could catch up.

When participants got a glimpse of how to control, they started to feel calm, to trust the machine, or even to build self-connection to it. They would touch, move closer, and feel confident to move inside the chamber. There were multiple reactions when the self-breathe function was activated. Some who had tight control over their breaths would notice the change. Some followed the machine breaths (i.e., synchrony) without noticing the change.

I observed that the participants kept trying and trying – constantly contesting control – throughout the transitions between the mirror and self-breathe functions. They paid a lot of attention to their bodies, their breaths, and their stomach movement, while keeping their gaze tight on the air pillows. During the interaction, the participant and I often chatted, discussing

‘What is happening now? Are you in control?’ Here, I noted that the poetic interaction emerged, as they could not distinguish when they were in control of the inflatables and when they were not. Some felt ‘disoriented’ or ‘pulled out of an immersion’ (P2, P16) if they had established a body-machine connection. Others described this contest as a positive experience of ‘having a dialogue’ with the machine (P5, P11, P15). P5 described, ‘I felt it [the machine] was telling me like, “Breathe, breathe.” And then maybe I did, right? As a consequence of that.’

I noted that some participants questioned their breaths and bodies within this feeling of half-control and half-loss-of-control. They became skeptical about their control over their bodies or if their bodies were working as they were supposed to. P7 explicitly said, ‘It made me wonder if I'm using my full lung capacity.’

Furthermore, I noticed that the user’s sense of control is not always the same as the user’s actual control. I observed that synchrony happened as intended (participants breathed in and out, following the machine’s self-breathe function), but many of them were not aware of it. Some saw that as a glitch in the system and that they would like to reset the machine to match their breaths again. Many others perceived that the machine on the self-breathe function “is now functioning”. This showed that most participants did not know if they were being manipulated or in control. When I informed them that the system had influenced/manipulated their breaths at times, most participants would reject and report that they did not experience so at all. They insisted that they connected with the machine and didn’t feel ‘controlled.’

Medical perspectives

Participants did not directly associate their experience to biomedical technologies or clinical experience, unless I prompted or asked for their association. Nevertheless, medical experts expressed the resemblance of the aesthetic experience to real-life medical experiences. P13 reckoned the agency of clinical apparatus that asserted discomfort to patients, ‘[Medical technology] is hard and sharp. […] It pushes and cuts.’ P15 associated the unpredictable behaviours of the installation with moments when ‘medical devices or medical practitioners don't behave the way you want them to behave’, which could lead to frustration. On the opposite end, P12 and P16 associated the work with the notion of ‘care.’ They perceived the soft touch from the air pillows as gentle and warm hugs, which echoes in health culture and even healthcare industries.

In critical reflection on medical practices, P13 and P19 reflected upon the negation of power, consent and submission to medical treatment. They reflected upon how consent request is part of the contemporary rhetoric of medicine. Yet, this rhetoric gives the sense of ‘sort of in control’ (P13), but it raises questions about an ‘illusion of agency’ (P19). P13 contemplated upon the topic of politics of control within the medical institutions in the present society. Besides, P19 reflected on the power which medical technologies hold, as a separate entity from medical practitioners. She found it ‘eye-opening’ to regard medical devices and apparatus as subjects with an agency. Finally, she further speculated on the requirements and conditions for patients to consciously surrender and willingly trust in the medical datafying system.

> *‘I think it's really interesting in terms of, you know, medical devices and how much can we fight back and can we say, “no, we're not going to do that.” Or maybe medicine should be a bit bolder and say, “no, you are going to do it” […] with vaccinations in mind. ’– P13*
>
> *‘Transparency and accessibility of data, like your data, especially seems to be the main factor that mediates trust in the [medical] system.’ – P19*



Figure 8: Several modes of interaction (connecting with the balloons through touch; letting oneself be squeezed in between the air pillows; playfully going around the air pillows to observe them)

Reflection and future perspectives

Contesting control with physiological sensors provided me a compelling framework for creators to both reflect and materialise the experience of fighting for agency between human, body and technology. In my process, the principle pointed my focus toward the notion of ambiguity and intricacies of autonomy in the clinical context (section 2). The offered vocabulary supported my investigation and evaluation of the artefact experience (section 4). It was my main guiding rail for creating a tangible system with aesthetic interaction (section 5). And the interviews with participants illustrated that the artefact could evoke a sense of grappling for control and entangled agency (section 7).

In detail, ambiguous control produced an ongoing, long-lasting grappling for control. This notion 'ongoing, long-lasting' made this project different from given example projects in contesting control where users just pass by this ambiguous control space for a short moment (Benford et al., 2021). The participants described the experience with Breathe Kritisk as a conversation or a dialogue that they have with the machine. They reported their experience as a constant play – a process to make sense of the world (Sicart, 2014). Ambiguous control created a mysterious scene for users to figure out how the system control works and if they can ever control the machine. During this play, the mystery heightened users' scepticism in control, and ambiguated their perception of control. Their belief in their control often tipped back and forth, which raised questions about their agency and their perception of agency. This notion adds another layer to the embodied experiences of "dance of agency" (Penny, 2011) with the "dance of perception in agency". I remark that this could be what future creators, artists and designers could aim for and possibly define ambiguous control.

Ambiguous control was a powerful medium for participatory experience that could provoke reflection around entangled agency between humans and technology. In our project, ambiguous control offered an embodied experience of cyborgs where machine body and living body are intertwined, as well as their agencies (Haraway, 2006). Living organisms assert control and influence on technologies, and vice versa. The experience of entanglement in the functional prototype led to some discussion around medical practices, but often limited to people with clinical/medical background. I assume the challenge could be the loose connection between the functional prototype to the medical context. Consequently, my future development steps include creating a sterile clinical aesthetic and adding a performative layer

with a role-play medical assistant and clinical props, to create a clearer relation to the digital clinical gaze for public visitors. I plan to exhibit the installation at an international design week in Malmö, Sweden and observe visitors/audience reactions to the installation in public. It is my interest to investigate if the audience could resonate with the intended experience and how they make sense of the interaction.

Nevertheless, I see that ambiguous control can be relevant for other practice-based research on other impending themes of artificial intelligence, data surveillance, more-than-human, and the politics of individual agency in a complex network. It can make abstract philosophy and concept of entangled agency physical, tangible and experiencable. Its experience can provoke reflection toward socially-given systems and open up rich discussion around the invisible power and hierarchy that is embedded in social norms.

ACKNOWLEDGEMENTS

The project was developed as a part of a PhD design research at IT University of Copenhagen in collaboration with Munch museum. AIR lab and Halfdan Hauch Jensen contributed greatly to the physical construction of the installation prototype. I am grateful to participants from IT University of Copenhagen, Medical Museion, and Technical University of Denmark for their valuable feedback.

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